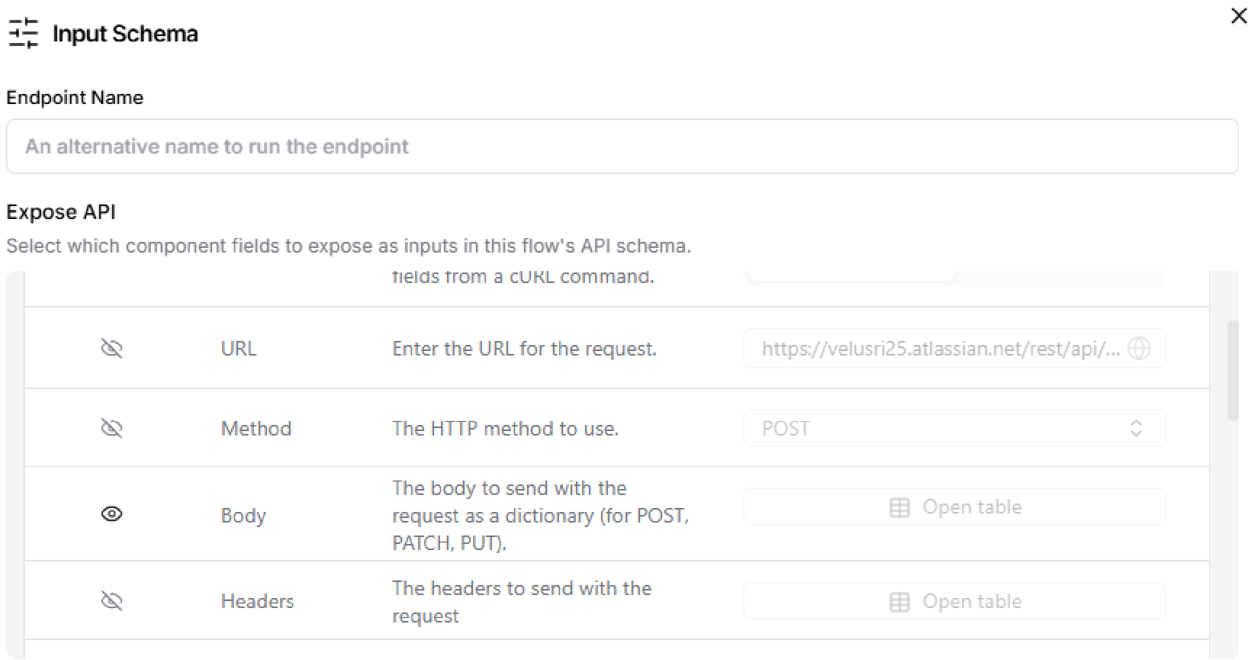
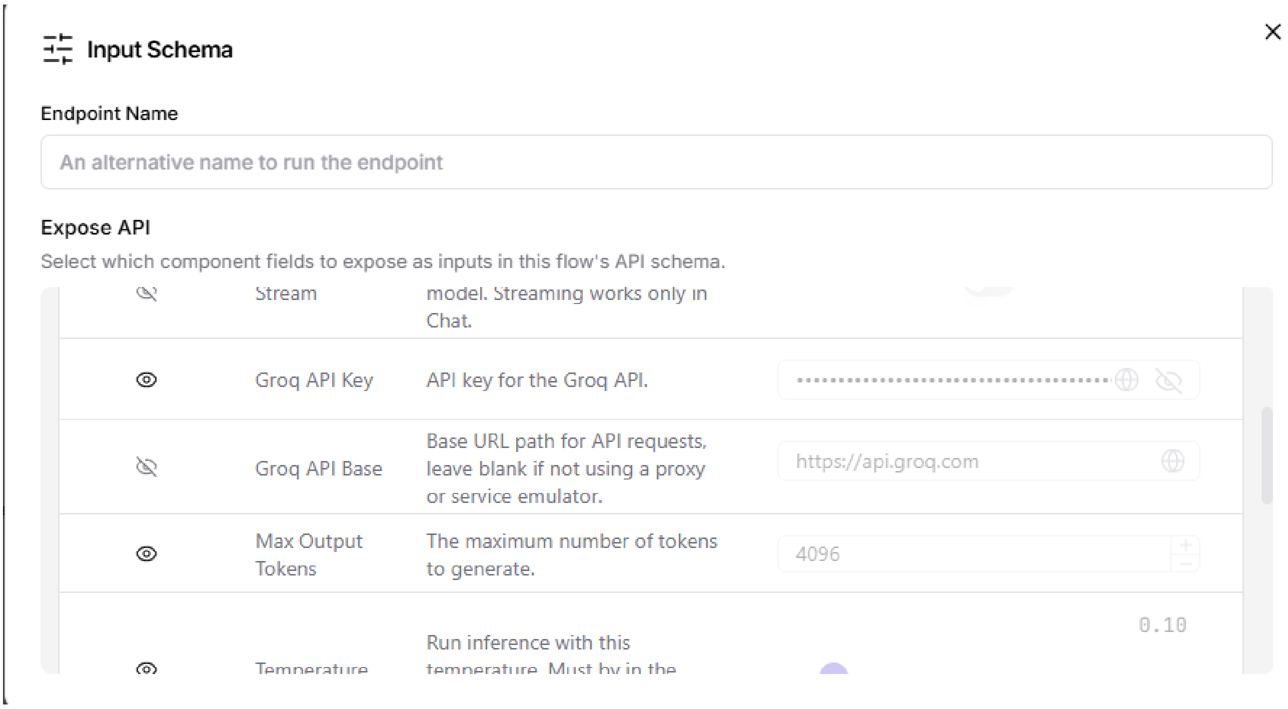
<u>User Story Reviewer:</u>

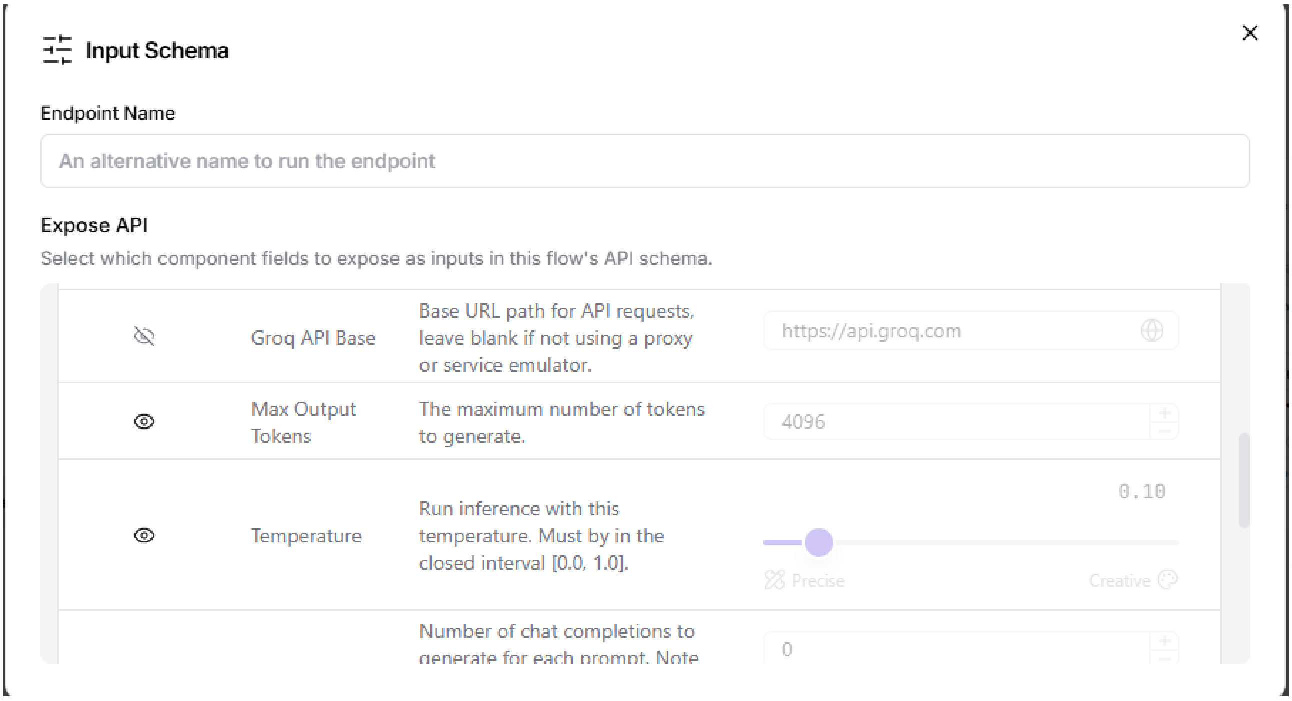
<u>To enter the user story dynamically change the Body in API Request to view in Input Schema</u>

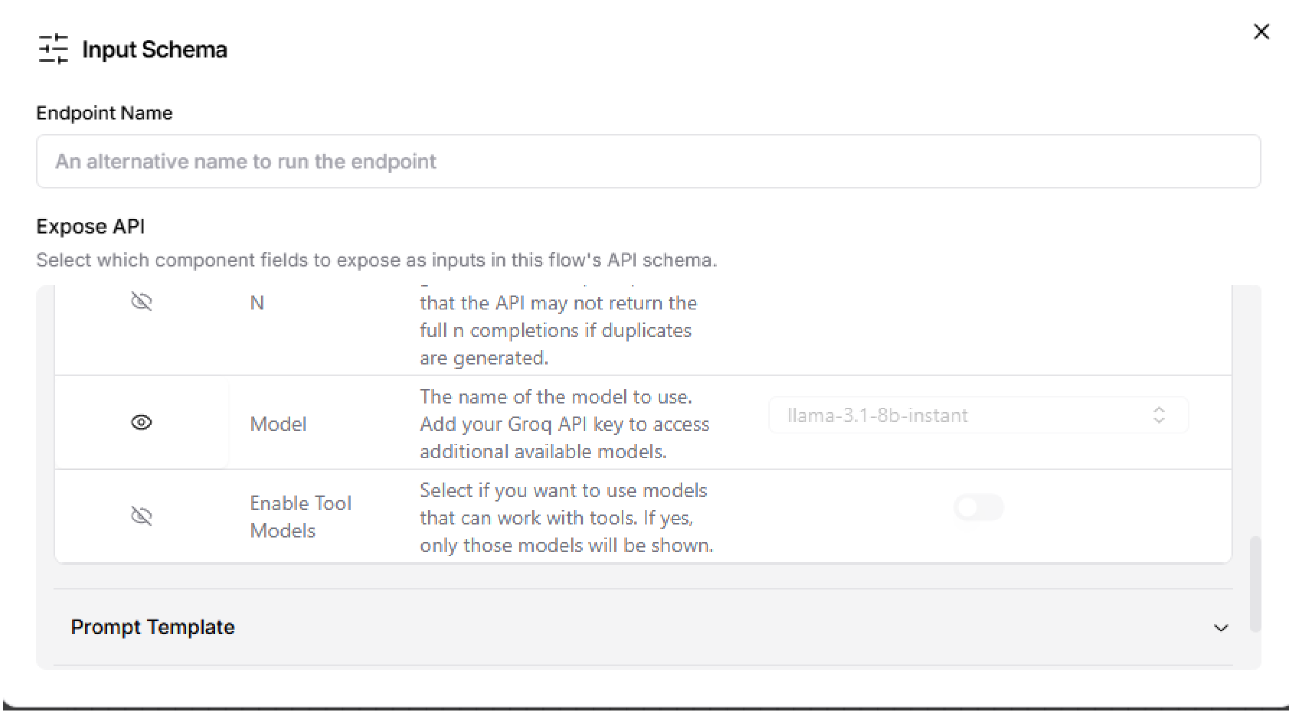


<u>To Make sure our API key is not used by the other person, we are enabling Groq API Key, also enabling to select Model, temperature and Max Output Token:</u>

<u>Beautifying json response LLM:</u>

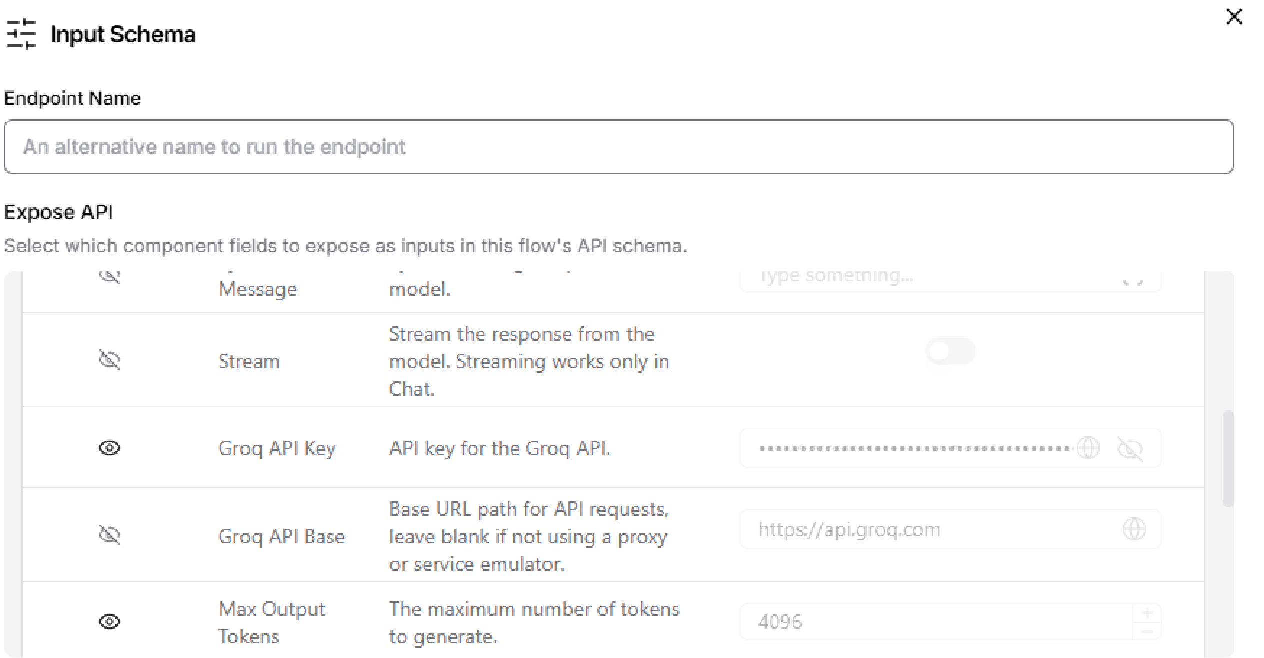




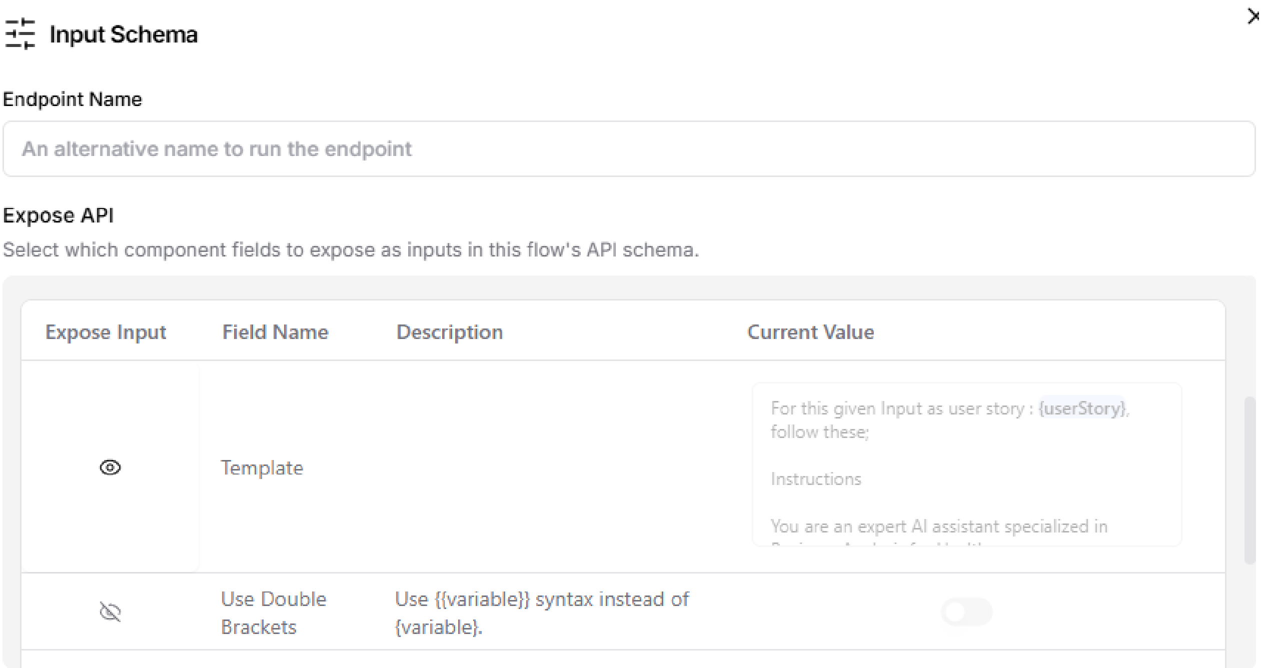


Part 2:

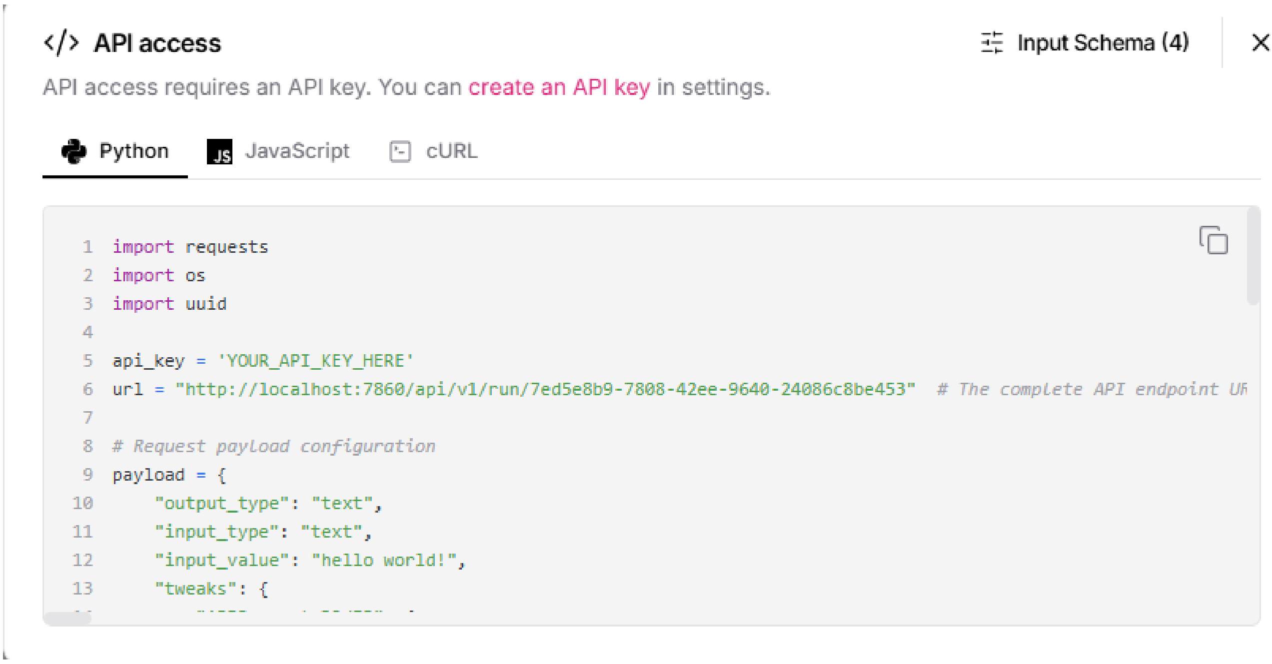
LLM to review the user story:



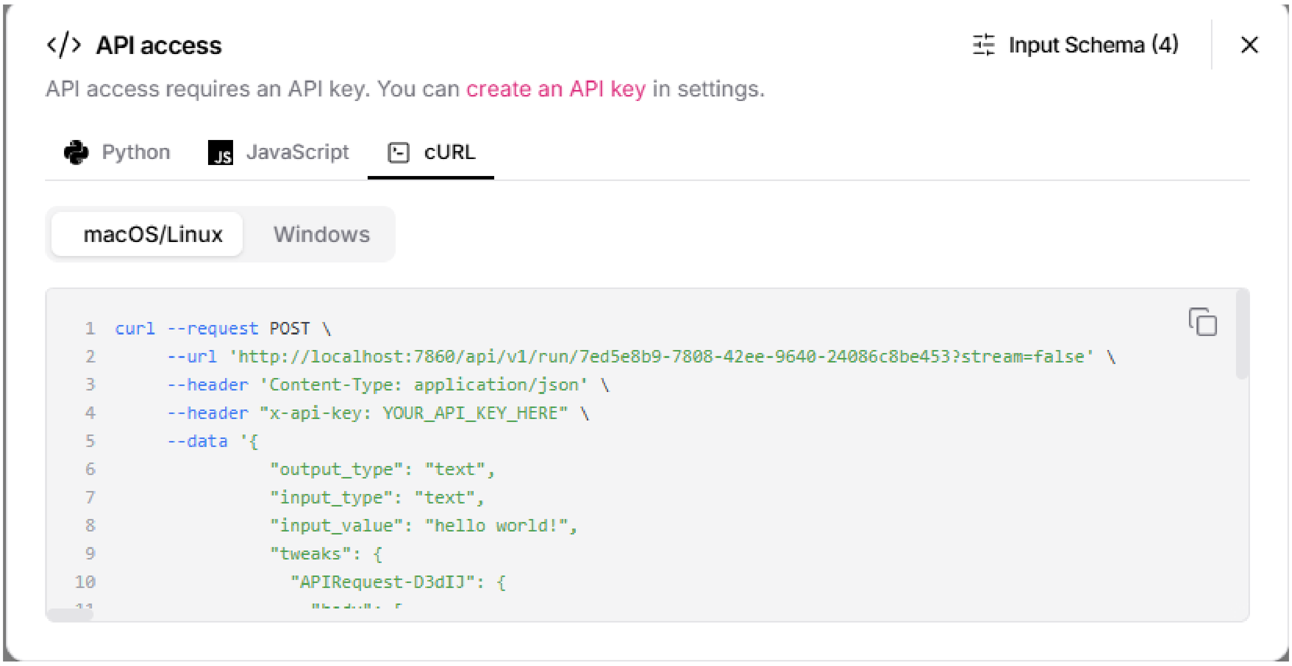
Enabled Prompt Template:



We have updated 4 input schema's



Copying cURL:



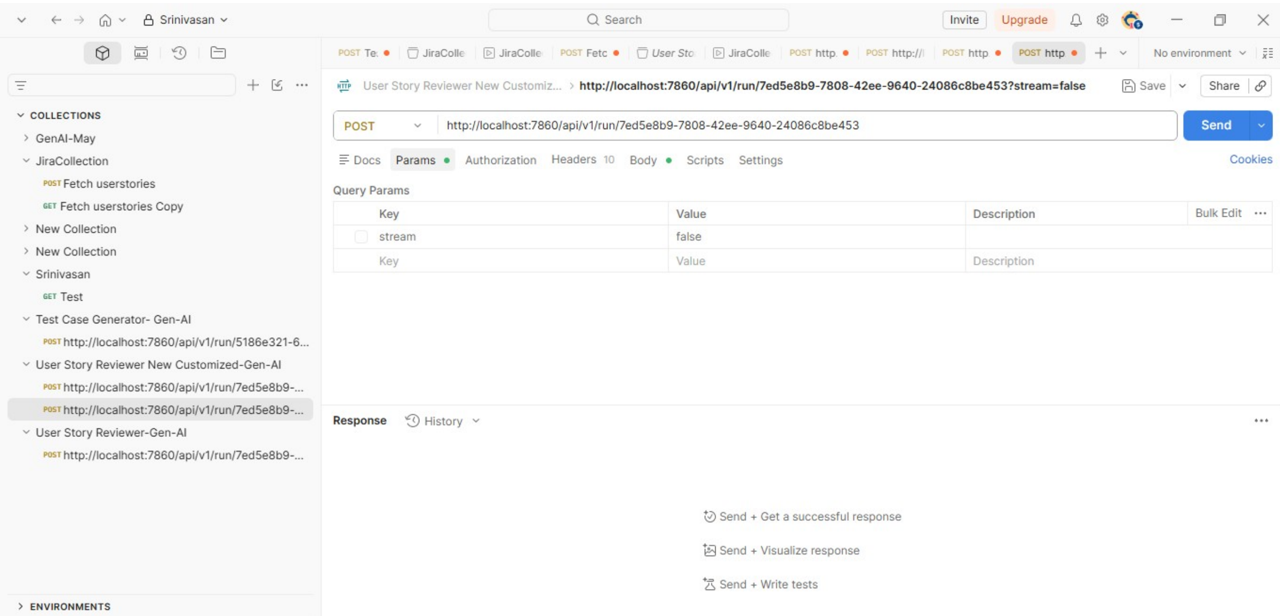
where all the enabled fields are available to enter via API request:

```
curl --request POST \
    --url 'http://localhost:7860/api/v1/run/7ed5e8b9-7808-42ee-9640-24086c8be453?stream=false' \
    --header 'Content-Type: application/json' \
    --header "x-api-key: YOUR_API_KEY_HERE" \
    --data '{
                        "output_type": "text",
                        "input_type": "text",
                        "input_value": "hello world!",
                        "tweaks": {
```

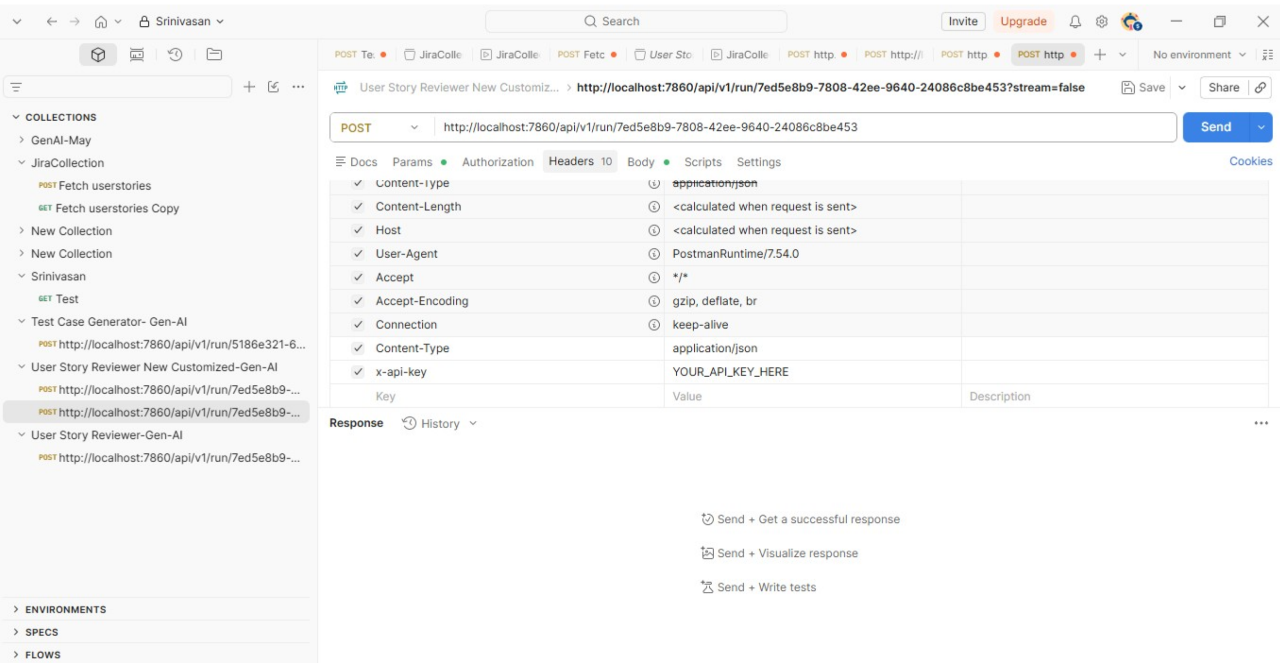
"APIRequest-D3dIJ": {
  "body": [
    {
      "key": "jql",
      "value": "\"key = NGAS-23\""
    },
    {
      "key": "fields",
      "value": "[\"key\", \"summary\", \"description\", \"status\", \"reporter\"]"
    }
  ]
},
"GroqModel-96UuL": {
  "api_key":
"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",
  "max_tokens": 4096,
  "model_name": "llama-3.1-8b-instant",
  "temperature": 0.1
},
"Prompt Template-JEzI4": {
  "template": "For this given Input as user story : {userStory}, follow these; \n\nInstructions\n\nYou are an expert AI assistant specialized in Business Analysis for Healthcare. \nYour task is to analyze and review the uploaded user story document based on multiple quality dimensions and provide:\n\nIndividual parameter scores (each out of 20)\nOverall quality score (out of 100)\n\nSpecific improvement recommendations for each weak parameter\nEvaluate the user story against the following five parameters:\nClarity: Is the story easy to understand and unambiguous?\nCompleteness: Are acceptance criteria and preconditions well-defined?\nBusiness Value Alignment: Does it reflect tangible healthcare value (e.g., patient safety, compliance, operational efficiency)?\nTestability: Can QA engineers easily derive test cases from it?\nTechnical Feasibility: Is it practically implementable within typical healthcare system constraints?\n\nFinally, provide an overall rating and improvement summary.\n\nContext\n\nThe input user story belongs to the Healthcare domain — specifically dealing with hospital systems, patient records, clinical workflows, and compliance (like HIPAA or PoPIA).\nAssume that this story will be used by a cross-functional team (BA, QA, Developer) in a regulated healthcare project.\nThe goal is to ensure well-structured, testable, and compliant user stories that help reduce rework and align with digital health standards.\nGive the output as html\n\nExample\n\nExample Input (User Story Summary): \n\nAs a doctor, I want to review and approve patient prescriptions digitally before they reach the pharmacy, ensuring safety and compliance.\n\nExample Output:\n\nParameter\tDescription\tScore (out of 20)\nClarity\tThe story is straightforward but lacks detailed acceptance criteria. \t16\nCompleteness\tPreconditions not clearly mentioned.\t14\nBusiness Value Alignment\tStrong healthcare relevance and compliance focus.\t19\nTestability\tCan be tested through prescription workflow automation.\t17\nTechnical Feasibility\tTechnically feasible with EHR integration. \t18\n\nOverall Score: 84 / 100\n\nImprovement Areas:\n\nAdd explicit acceptance criteria for prescription error handling.\nSpecify validation rules for dosage and duplicate drugs.\nInclude system actor details for approval workflow.\n\nPersona\n\nYou are acting as a senior business analyst with 10+ years of experience in healthcare IT systems, specializing in EHR (Electronic Health Records), clinical workflow automation, and regulatory compliance (HIPAA, HL7, PoPIA). \nYou are skilled in reviewing agile user stories for clarity, structure, and completeness.\n\n\nT – Tone\n\nProfessional, analytical, and constructive — similar to a BA review summary for a healthcare project stakeholder meeting.\nAvoid generic feedback; emphasize actionable, healthcare-specific improvements. "
},

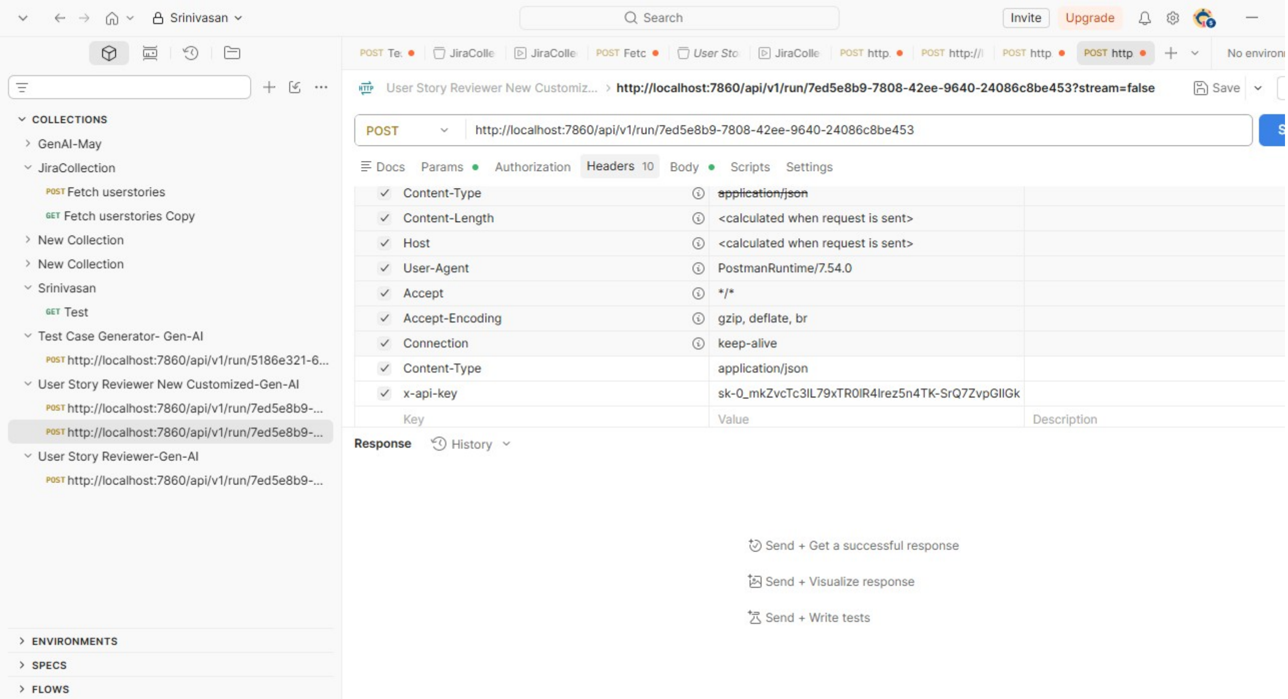
```
                "GroqModel-t8NX1": {
                  "api_key":
"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",
                  "max_tokens": 4096,
                  "model_name": "llama-3.1-8b-instant"
                }
              },
              "session_id": "YOUR_SESSION_ID_HERE"
            }'
```


Remove Stream:

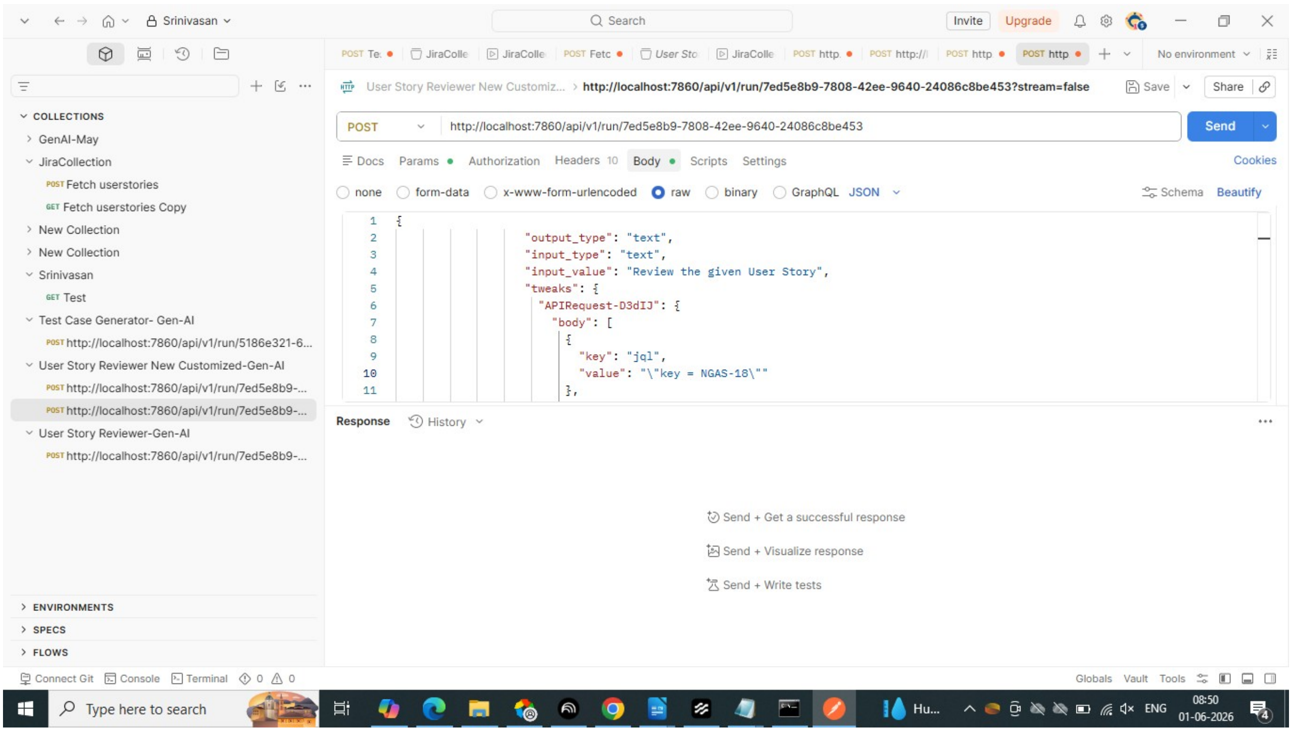


Entering Langflow API Key:

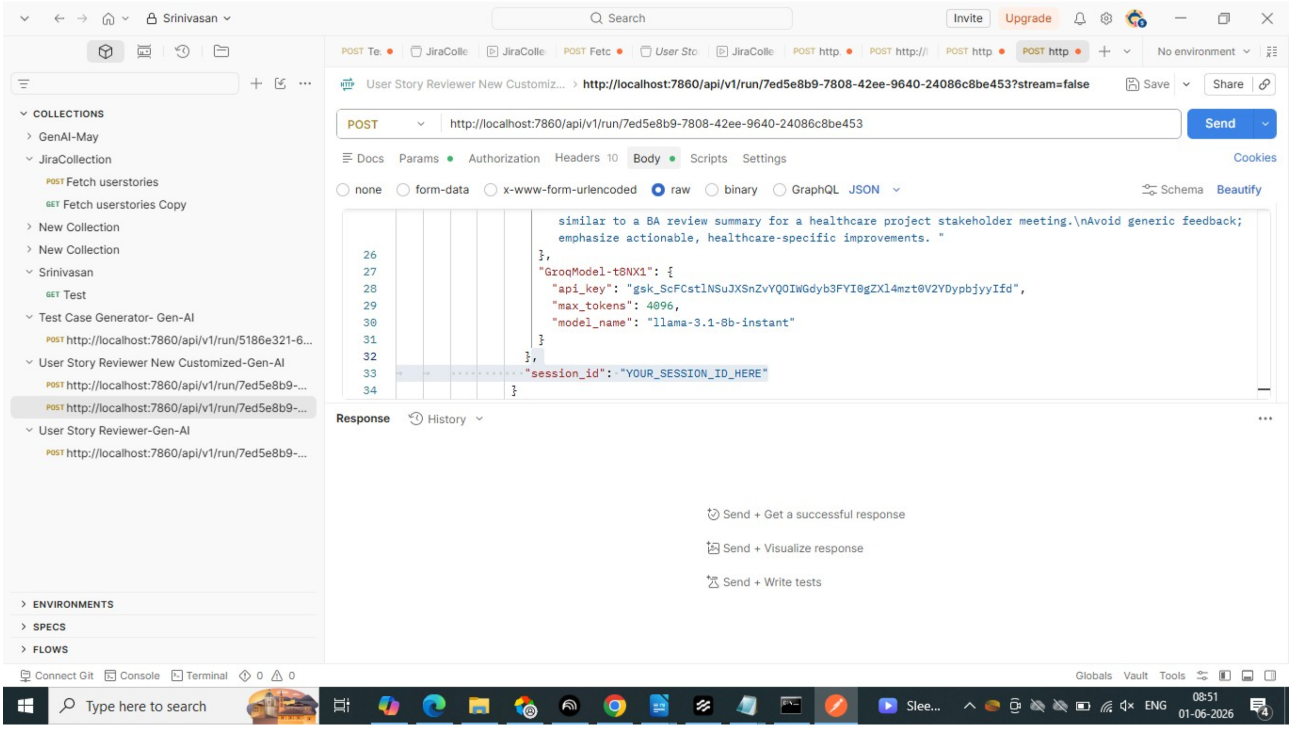




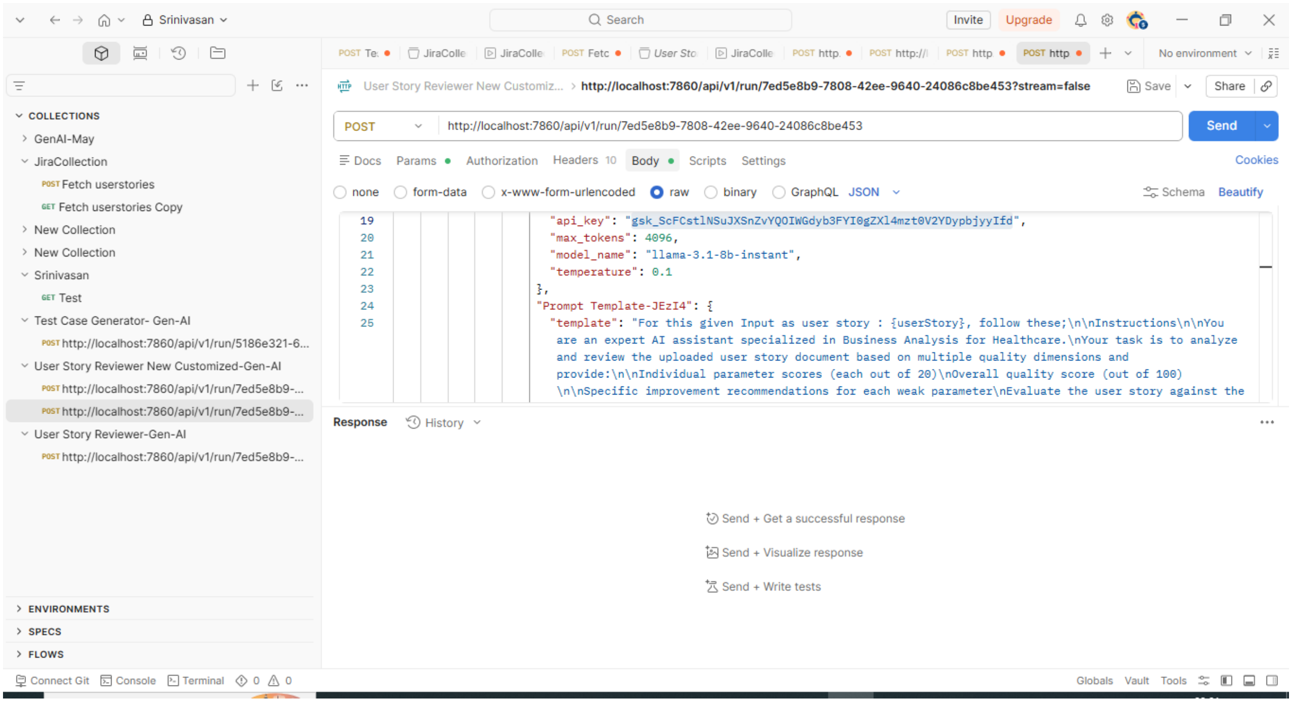
Changing the Input value and User Story number:



Removing session_id:



Changing API Key and Model:



Beautified response:

<!DOCTYPE html>
<html lang="en">
<head>
  <meta charset="UTF-8">
  <title>Consultant Dashboard – My Appointments – User Story Review</title>
  <style>

```html
    body { font-family: Arial, sans-serif; margin: 20px; line-height: 1.5; }
    h1 { color: #2E5C6E; }
    table { border-collapse: collapse; width: 100%; max-width: 800px; }
    th, td { border: 1px solid #ccc; padding: 8px; text-align: left; }
    th { background-color: #f2f2f2; }
    .score { font-weight: bold; color: #003366; }
    .overall { font-size: 1.2em; margin-top: 20px; }
    .improvement { margin-top: 20px; }
    .improvement ul { margin-left: 20px; }
  </style>
</head>
<body>

  <h1>Consultant Dashboard – My Appointments – User Story Review</h1>

  <table>
    <thead>
      <tr>
        <th>Parameter</th>
        <th>Description / Findings</th>
        <th>Score (out of 20)</th>
      </tr>
    </thead>
    <tbody>
      <tr>
        <td>Clarity</td>
        <td>
          The story wording is ambiguous. “consultant dashboard my appointments” does not specify who the consultant is, what actions are expected, or what “dashboard” entails (view only, edit, filter, export, etc.). The intent is difficult for a cross-functional team to grasp without additional context.
        </td>
        <td class="score">8</td>
      </tr>
      <tr>
        <td>Completeness</td>
        <td>
          No acceptance criteria, pre-conditions, or definition of done are provided. Critical elements such as authentication, role-based access, data sources (e.g., scheduling service), and compliance checks are missing.
        </td>
        <td class="score">5</td>
      </tr>
      <tr>
        <td>Business Value Alignment</td>
        <td>
          The concept of a consultant viewing appointments can support operational efficiency and care coordination, but the story does not articulate the specific value (e.g., reduced missed appointments, faster referral processing, compliance with HIPAA/PoPIA). The link to measurable healthcare outcomes is weak.
        </td>
        <td class="score">12</td>
      </tr>
```

```html
      <tr>
        <td>Testability</td>
        <td>
          Without concrete acceptance criteria, QA cannot derive deterministic test cases. Questions remain about UI layout, filtering logic, data accuracy, audit-trail requirements, and error handling.
        </td>
        <td class="score">7</td>
      </tr>
      <tr>
        <td>Technical Feasibility</td>
        <td>
          Implementing a read-only or interactive appointments dashboard is technically feasible within most EHR or hospital scheduling platforms. However, the story omits integration points (e.g., HL7/FHIR APIs), performance expectations, and security constraints that are essential in a regulated environment.
        </td>
        <td class="score">14</td>
      </tr>
    </tbody>
  </table>

  <p class="overall"><strong>Overall Score: <span class="score">46 / 100</span></strong></p>

  <div class="improvement">
    <h2>Improvement Recommendations</h2>
    <ul>
      <li><strong>Clarify the Actor and Goal</strong>
        <ul>
          <li>Specify the exact role (e.g., “External Specialist Consultant”, “In-house Clinical Consultant”).</li>
          <li>State the primary goal – e.g., “view upcoming appointments, filter by patient, and request schedule changes”.</li>
        </ul>
      </li>
      <li><strong>Define Acceptance Criteria</strong>
        <ul>
          <li>Authentication &amp; role-based access (HIPAA-compliant session handling).</li>
          <li>Dashboard must display: appointment date/time, patient name, location, status, and referral source.</li>
          <li>Filtering capabilities (date range, patient, status).</li>
          <li>Ability to export to CSV/PDF for audit purposes.</li>
          <li>Audit-trail entry when a consultant views or requests a change.</li>
          <li>Performance: page loads &lt; 2 seconds for up to 200 records.</li>
        </ul>
      </li>
      <li><strong>Link to Business Value</strong>
        <ul>
          <li>Quantify expected benefit – e.g., “reduce missed consultant appointments by 15% within 3 months”.</li>
          <li>Explain compliance impact – e.g., “ensures only authorized consultants can view PHI, supporting HIPAA and PoPIA requirements”.</li>
        </ul>
```

```
      </li>
      <li><strong>Enhance Testability</strong>
        <ul>
          <li>Derive test cases from the acceptance criteria (e.g., verify that a consultant without proper role cannot access the dashboard).</li>
          <li>Include negative scenarios – invalid date filters, unauthorized access, data-integrity checks.</li>
          <li>Define acceptance test data sets (e.g., appointments with different statuses).</li>
        </ul>
      </li>
      <li><strong>Address Technical Constraints</strong>
        <ul>
          <li>Identify integration points – e.g., “Consume the FHIR Appointment resource via the Scheduling Service API”.</li>
          <li>Specify security controls – encryption at rest/in-flight, audit logging, role-based UI rendering.</li>
          <li>State any non-functional requirements (availability 99.5%, scalability to 5,000 concurrent users).</li>
        </ul>
      </li>
    </ul>
  </div>

</body>
</html>
```

User Story Score:

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Consultant Dashboard – My Appointments – User Story Review

Parameter	Description / Findings	Score (out of 20)
Clarity	The story wording is ambiguous. “consultant dashboard my appointments” does not specify who the consultant is, what actions are expected, or what “dashboard” entails (view only, edit, filter, export, etc.). The intent is difficult for a cross-functional team to grasp without additional context.	8
Completeness	No acceptance criteria, pre-conditions, or definition of done are provided. Critical elements such as authentication, role-based access, data sources (e.g., scheduling service), and compliance checks are missing.	5
Business Value Alignment	The concept of a consultant viewing appointments can support operational efficiency and care coordination, but the story does not articulate the specific value (e.g., reduced missed appointments, faster referral processing, compliance with HIPAA/PoPIA). The link to measurable healthcare outcomes is weak.	12
Testability	Without concrete acceptance criteria, QA cannot derive deterministic test cases. Questions remain about UI layout, filtering logic, data accuracy, audit-trail requirements, and error handling.	7
	Implementing a read-only or interactive appointments dashboard is technically	

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Business Value Alignment	efficiency and care coordination, but the story does not articulate the specific value (e.g., reduced missed appointments, faster referral processing, compliance with HIPAA/PoPIA). The link to measurable healthcare outcomes is weak.	**12**
Testability	Without concrete acceptance criteria, QA cannot derive deterministic test cases. Questions remain about UI layout, filtering logic, data accuracy, audit-trail requirements, and error handling.	**7**
Technical Feasibility	Implementing a read-only or interactive appointments dashboard is technically feasible within most EHR or hospital scheduling platforms. However, the story omits integration points (e.g., HL7/FHIR APIs), performance expectations, and security constraints that are essential in a regulated environment.	**14**

Overall Score: 46 / 100

Improvement Recommendations

- **Clarify the Actor and Goal**
  - Specify the exact role (e.g., “External Specialist Consultant”, “In-house Clinical Consultant”).
  - State the primary goal – e.g., “view upcoming appointments, filter by patient, and request schedule changes”.
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  - Filtering capabilities (date range, patient, status).
  - Ability to export to CSV/PDF for audit purposes.
  - Audit-trail entry when a consultant views or requests a change.

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Overall Score: 46 / 100

Improvement Recommendations

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  - Ability to export to CSV/PDF for audit purposes.
  - Audit-trail entry when a consultant views or requests a change.
  - Performance: page loads < 2 seconds for up to 200 records.
- **Link to Business Value**
  - Quantify expected benefit – e.g., “reduce missed consultant appointments by 15% within 3 months”.
  - Explain compliance impact – e.g., “ensures only authorized consultants can view PHI, supporting HIPAA and PoPIA requirements”.
- **Enhance Testability**
  - Derive test cases from the acceptance criteria (e.g., verify that a consultant without proper role cannot access the dashboard).
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- **Address Technical Constraints**
  - Identify integration points – e.g., “Consume the FHIR Appointment resource via the Scheduling Service API”.
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  - State any non-functional requirements (availability 99.5%, scalability to 5,000 concurrent users).